

Start of Care Assessment, Pediatric

CCG: C-1110 (CCG)

MCG Health
Chronic Care
28th Edition

- Assessment
 - Language
 - Hearing
 - Vision
 - Health Status
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Assessment

Language

Q1. Patient: Do you speak English?

- Yes
- No [Q2] [Q3]
- Unknown [Q2] [Q3]

Q2. Case Manager: What language does the patient speak?

- Arabic [B1]
- Armenian [B1]
- Cambodian [B1]
- Chinese [B1]
- Croatian [B1]
- Czech [B1]
- Danish [B1]
- Dutch [B1]
- Filipino (Tagalog) [B1]
- Finnish [B1]
- Formosan [B1]
- French [B1]
- French Creole [B1]
- German [B1]
- Greek [B1]
- Hebrew [B1]
- Hindi [B1]
- Hmong [B1]
- Hungarian [B1]
- Italian [B1]
- Japanese [B1]
- Korean [B1]
- Lithuanian [B1]
- Navaho [B1]
- Norwegian [B1]
- Persian [B1]
- Polish [B1]

- Portuguese [B1]
- Punjabi [B1]
- Romanian [B1]
- Russian [B1]
- Serbo-Croatian [B1]
- Slovak [B1]
- Spanish [B1]
- Swedish [B1]
- Thai (Laotian) [B1]
- Turkish [B1]
- Ukrainian [B1]
- Vietnamese [B1]
- Yiddish [B1]
- Other, specify: _____ [B1]
- Unknown [B1]

Q3. Patient: Is there anyone available who can translate?

- Yes, the patient's family or caregiver is available and speaks and understands English. [Q4] [Q5] [B1]
- Yes, the patient's family or caregiver speaks and understands English, but arrangements must be made for them to be available. [Q4] [Q5] [B1]
- No [Q6] [B1] [B2] [P1]
- Unknown [Q6] [B1] [B2] [P1]

Q4. Caregiver: Is the patient OK with you speaking on their behalf?

- Yes [Q5]
- No [Q6] [P1]
- Unknown [Q6] [P1]

Q5. Patient: Are there problems with reading or writing?

- Yes [B1] [P1] Health Literacy  CCG
- No
- Unknown [P1] Health Literacy  CCG

Q6. Patient: Is it OK to use a translation service?

- Yes
- No [B2] [B1] [P1]
- Unknown [B2] [B1] [P1]
- Not applicable

Hearing

Q1. Caregiver: Does your child have a hearing problem?

- Yes [Q2] [Q3] [Q4] [B3]
- No
- Unknown [Q2] [Q3] [Q4]

Q2. Caregiver: Does your child see a specialist ear or hearing doctor (otolaryngologist)?

- Yes [P2]
- No [P2] [P3]
- Unknown [P2]

Q3. Caregiver: Does your child use any tools to make up for hearing loss? (Check all that apply.)

- Hearing aids
- Sign language
- Sign language interpreter
- Telecommunications relay service (TRS) (eg, TTY or TDD)
- Writing notes
- Other, specify: _____ [P2]
- None [P2] [P3]
- Unknown [P2]

Q4. Caregiver: Do you have problems caring for your child due to the hearing loss?

- Yes [P2] [P3]
- No
- Unknown [P2] [P3]

Vision**Q1. Caregiver:** Does your child have problems with their vision?

- Yes, specify: _____ [Q2] [Q3] [Q4] [B4] [P4]
- No
- Unknown [Q2] [Q3] [Q4] [P5]

Q2. Caregiver: Does your child see a specialist eye doctor for their vision problems?

- Yes [P4]
- No [P4] [P5]
- Unknown [P5]

Q3. Caregiver: What tools does your child use to make up for vision loss? (Check all that apply.)

- Braille [P4]
- Computer software [P4]
- Glasses or contact lenses [P4]
- Global Positioning System (GPS) for the visually impaired [P4]
- Magnifying lenses [P4]
- Sighted companion [P4]
- White cane or mobility training [P4]
- Other, specify: _____ [P4]
- None [P4] [P5]
- Unknown [P4]

Q4. Caregiver: Do you have trouble caring for your child because of their vision problems?

- Yes [P4] [P5]
- No
- Unknown [P4] [P5]

Health Status**Q1. Caregiver:** Please tell me about your child's current health. What is your child's main health problem? When was it diagnosed? (Check all that apply.)

- Asthma, date diagnosed: _____
- Cancer, date diagnosed: _____
- Cystic fibrosis, date diagnosed: _____
- Diabetes, date diagnosed: _____
- Eating disorder, date diagnosed: _____
- HIV/AIDS, date diagnosed: _____
- Infant premature or child with special healthcare needs, date diagnosed: _____
- Obesity, date diagnosed: _____
- Sickle cell disease, date diagnosed: _____
- Spinal cord injury, date diagnosed: _____
- Traumatic brain injury, date diagnosed: _____
- Other, specify: _____
- Unknown

Q2. Caregiver: Comorbid Conditions: What other health problems does your child have? (Check all that apply.)

- Arthritis
- Asthma
- Blood clotting problems
- Cancer
- Cardiac congenital defects
- Chronic obstructive pulmonary disease (COPD)
- Coronary artery disease
- Cystic fibrosis
- Diabetes
- Heart failure
- High blood pressure

- Infections or immune system problems
- Kidney failure
- Liver disease
- Neurologic problems
- Obesity or overweight
- Osteoporosis or degenerative bone disorder
- Seizures
- Stroke history
- Traumatic brain injury
- Other, specify: _____
- None

Q3. Caregiver: Does your child have any behavioral health problems? If yes, what are they? (Check all that apply.)

- Anxiety
- Attention-deficit hyperactivity disorder (ADHD)
- Autism spectrum disorder
- Bipolar disorder
- Depression
- Schizophrenia
- Substance use disorder
- Other, specify: _____
- None
- Unknown

Q4. Caregiver: Treatment History: Please describe your child's treatment for the main health problem. I'm interested in medicines, hospital stays, procedures, and other treatments.

- Treatment history: _____
- Unknown

Q5. Caregiver: Hospital Inpatient Stays: Has your child ever stayed overnight in a hospital? If so, when and for what reason?

- Hospital inpatient stays: _____ [Q6]
- No
- Unknown

Q6. Caregiver: Has your child been to the emergency room since last being in the hospital?

- Yes, when and why: _____
- No
- Unknown

Q7. Caregiver: Does your child have a primary doctor (such as a pediatrician or family doctor)?

- Yes, name and number: _____
- No [P6]
- Unknown [P6]

Q8. Caregiver: Does your child get healthcare from someone besides the doctor (such as healers, herbalists, or acupuncturists)?

- Yes, specify: _____
- No
- Unknown

Q9. Caregiver: Does your child have any medical allergies (such as to medicine or latex)?

- Yes, specify: _____
- No
- None known
- Unknown

Q10. Caregiver: What vaccinations has your child had? (Check all that apply.) **QM**

- Coronavirus disease 2019 (COVID-19) [Q11]
- *Haemophilus influenzae* type b (Hib) [Q11] [Q12]
- Hepatitis A [Q11]
- Hepatitis B [Q11]
- Human papillomavirus (HPV) [Q11]
- Influenza [Q11] [Q12]
- Measles, mumps, rubella (MMR) [Q11]
- Meningococcal 4-valent conjugate (MenACWY) or polysaccharide (MPSV4) [Q11]

- Meningococcal B (MenB) [Q11]
- Pneumococcal 13-valent conjugate (PCV13) [Q11]
- Pneumococcal 15-valent conjugate (PCV15) [Q11]
- Pneumococcal 20-valent conjugate (PCV20) [Q11]
- Pneumococcal 23-valent polysaccharide (PPSV23) [Q11]
- Polio (inactivated) [Q11]
- Respiratory syncytial virus (RSV) [Q11]
- Rotavirus [Q11]
- Tetanus booster [Q11]
- Tetanus, diphtheria, pertussis (DTaP/DT, Tdap/Td) [Q11]
- Varicella [Q11]
- Other, specify: _____ [Q11]
- None, because: _____ [P7]
- Unknown [P7]

Q11. Caregiver: Is your child up to date with the vaccinations needed (for age, booster status, or medical condition)? **QM**

- Yes
- No, because: _____ [Q13] [P7]
- Unknown [Q13] [P7]

Q12. Caregiver: When was your child's last flu shot? **QM**

- In the past year
- More than a year ago [Q13] [P7]
- Never [Q13] [P7]
- Unknown [Q13] [P7]

Q13. Caregiver: Do you have an appointment scheduled for your child to get caught up with their vaccinations?

- Yes
- No [P7]
- Unknown [P7]

Q14. Caregiver: How would you describe your child's health in the past year?

- Much better
- Somewhat better
- About the same
- Somewhat worse [P6]
- Much worse [P6]

Q15. Caregiver: How much does your child weigh and how tall is your child?

- Weight: _____
- Height: _____
- Unknown [P8]

Q16. Caregiver: What does your child eat and drink in a typical day? (Check all reported.)

- Alcohol, specify types and number of daily servings: _____ [Q17] [Q18] [Q19] [Q20]
- Dairy foods, specify types and number of daily servings: _____ [Q17] [Q18] [Q19] [Q20]
- Fats, specify types and number of daily servings: _____ [Q17] [Q18] [Q19] [Q20]
- Fruit juices, specify types and number of daily servings: _____ [Q17] [Q18] [Q19] [Q20]
- Fruits, specify types and number of daily servings: _____ [Q17] [Q18] [Q19] [Q20]
- Meats (beef, chicken, fish), specify types and number of daily servings: _____ [Q17] [Q18] [Q19] [Q20]
- Milk, specify types and number of daily servings: _____ [Q17] [Q18] [Q19] [Q20]
- Sodas, specify types and number of daily servings: _____ [Q17] [Q18] [Q19] [Q20]
- Supplements, specify types and number of daily servings: _____ [Q17] [Q18] [Q19] [Q20]
- Sweets, specify types and number of daily servings: _____ [Q17] [Q18] [Q19] [Q20]
- Vegetables, specify types and number of daily servings: _____ [Q17] [Q18] [Q19] [Q20]
- Water, specify number of daily servings: _____ [Q17] [Q18] [Q19] [Q20]
- Whole grains, specify number of daily servings: _____ [Q17] [Q18] [Q19] [Q20]
- Other, specify: _____ [Q17] [Q18] [Q19] [Q20] [P8] [P9]
- No food by mouth [Q21] [P8] [P9]
- Unknown [Q17] [Q18] [Q19] [Q20] [P8] [P9]

Q17. Caregiver: How often does your child eat each day? (Check all reported.)

- 4 meals or more [P8] [P9]
- 2 or 3 meals
- 1 meal [P8] [P9]
- 0 meals [P8] [P9]
- 2 snacks or more [P8] [P9]
- 1 snack
- 0 snacks
- Other, specify: _____ [P8] [P9]
- Not applicable
- Unknown [P8] [P9]

Q18. Caregiver: How much fluid (such as water, juice, coffee, tea, milk) does your child have per day?

- Less than 3 cups (720 mL) [P8] [P9]
- 3 to 5 cups (720 mL to 1.2 L) [P8] [P9]
- More than 5 cups (1.2 L)
- Unknown [P8] [P9]

Q19. Caregiver: Does your child's doctor want your child to follow a special diet program? (Check all that apply.)

- Yes, DASH (low sodium) Diet Knowledge Insufficient - DASH Diet [CCG](#)
- Yes, heart healthy Diet Knowledge Insufficient - Heart Healthy [CCG](#)
- Yes, kidney diet Diet Knowledge Insufficient - Renal Dysfunction [CCG](#)
- Yes, low carb or low glycemic index Diet Knowledge Insufficient - Low Glycemic Index [CCG](#)
- Yes, pregnancy diet Diet Knowledge Insufficient - Pregnancy [CCG](#)
- Yes, other: _____ [P8]
- No
- Unknown [P8] [P9]

Q20. Caregiver: Do you or your child think your child needs to gain or lose weight?

- Gain [P8] [P9]
- Lose [P8] [P9]
- Neither
- Unknown [P8] Growth and Development, Pediatric [CCG](#)

Q21. Caregiver: Does your child get nutritional treatments or have an altered diet? (Check all that apply.)

- Behavioral health provider seen, specify: _____ [P9]
- Dental soft diet, specify: _____ [P9]
- Dietitian seen or following, specify: _____ [P9]
- Diet supplements, specify: _____ [P9]
- Enteral nutrition (tube feeding), specify: _____ [P9] Enteral Nutrition Management [CCG](#)
- IV fluids (hydration), specify: _____ [P9]
- Liquid diet, specify: _____ [P9]
- Mechanical soft diet, specify: _____ [P9]
- Parenteral nutrition, specify: _____ [P9] Parenteral Nutrition Management [CCG](#)
- Procedure(s) done by a doctor in office, specify: _____ [P9]
- Pureed diet, specify: _____ [P9]
- Speech-language pathology (SLP), specify: _____ [P8]
- Supervised during nutritional intake, specify: _____ [P8]
- Thickened liquid diet, specify: _____ [P9]
- Other, specify: _____ [P9]
- No
- Unknown [P8] [P9]

Functional Status

Q1. Caregiver: Does your child's health limit their activities of daily living, such as bathing, dressing, feeding or eating, or moving around without help?

- Yes Assistive Devices [CCG](#) Safety [CCG](#)

- No
- Unknown Assistive Devices [CCG](#) [Safety](#) [CCG](#)

Q2. Caregiver: Does your child's health limit their ability to learn or develop?

- Yes Growth and Development, Pediatric [CCG](#)
- No
- Unknown Growth and Development, Pediatric [CCG](#)

Q3. Caregiver: Does your child need to use assistive devices (either to care for themselves or to receive care by someone else)?

- Yes Assistive Devices [CCG](#) [Safety](#) [CCG](#)
- No
- Unknown Assistive Devices [CCG](#) [Safety](#) [CCG](#)

Q4. Caregiver: Does your child have problems sleeping?

- Yes Sleep [CCG](#)
- No
- Unknown

Q5. Caregiver: Does your child have pain?

- Yes Pain Assessment [CCG](#)
- No
- Unknown Pain Assessment [CCG](#)

Patient Care

Q1. Caregiver: What conditions would you or your child like help with managing? (Check all that apply.)

- Asthma Asthma, Pediatric - Self-Care [CCG](#)
- Attention-deficit hyperactivity disorder (ADHD) Attention-Deficit Hyperactivity Disorder, Pediatric - Self-Care [CCG](#)
- Autism spectrum disorder Autism Spectrum Disorders, Pediatric - Self-Care [CCG](#)
- Cardiac defects Children with Special Healthcare Needs - Self-Care [CCG](#)
- Cerebral palsy Children with Special Healthcare Needs - Self-Care [CCG](#)
- Craniofacial deformity Children with Special Healthcare Needs - Self-Care [CCG](#)
- Cystic fibrosis Cystic Fibrosis, Pediatric - Self-Care [CCG](#)
- Diabetes Diabetes, Pediatric - Self-Care [CCG](#)
- Down syndrome Children with Special Healthcare Needs - Self-Care [CCG](#)
- Eating disorder Eating Disorders, Pediatric - Self-Care [CCG](#)
- End-of-life care End-of-Life Care - Self-Care [CCG](#)
- Hydrocephalus Children with Special Healthcare Needs - Self-Care [CCG](#)
- Myelomeningocele or neural tube deficit Children with Special Healthcare Needs - Self-Care [CCG](#)
- Neonatal care Neonatal Care - Self-Care [CCG](#)
- Neuromuscular disorder Children with Special Healthcare Needs - Self-Care [CCG](#)
- Obesity Obesity, Pediatric - Self-Care [CCG](#)
- Palliative care Palliative Care - Self-Care [CCG](#)
- Sickle cell disease Sickle Cell Disease, Pediatric - Self-Care [CCG](#)
- Other, specify: _____
- Unknown

Medication

Q1. Caregiver: What are the names, doses, routes, and frequencies of all the medicines the patient takes? (Include nonprescription medicines.)

- No medicine taken
- Medicine, dose, route, frequency: _____
- Medicine, dose, route, frequency: _____
- Medicine, dose, route, frequency: _____
- Medicine, dose, route, frequency: _____

- Medicine, dose, route, frequency: _____
- Medicine, dose, route, frequency: _____
- Medicine, dose, route, frequency: _____
- Medicine, dose, route, frequency: _____
- Medicine, dose, route, frequency: _____
- Medicine, dose, route, frequency: _____
- Medicine, dose, route, frequency: _____
- Medicine, dose, route, frequency: _____
- Medicine, dose, route, frequency: _____
- Medicine, dose, route, frequency: _____
- Medicine, dose, route, frequency: _____
- Medicine, dose, route, frequency: _____
- Medicine, dose, route, frequency: _____
- Medicine, dose, route, frequency: _____
- Medicine, dose, route, frequency: _____
- Unknown [P10]

Q2. Caregiver: Do you know what each of these medicines is for?

- Yes
- No [P10]
- Unknown [P10]
- Not applicable

Q3. Caregiver: Is the patient taking any new medicines prescribed by the doctor?

- Yes, specify: _____ [P11]
- No, patient does not have any new medicine.
- No, patient hasn't started taking the new medicine. Medication Adherence [CCG](#)
- Unknown Medication Adherence [CCG](#)

Q4. Caregiver: Is the patient taking any new medicines that do not need a prescription (eg, over-the-counter medicines, vitamins, minerals, or herbal or dietary supplements)?

- Yes, specify: _____ [P11]
- No
- Unknown

Q5. Caregiver: Are there any medicines that the patient is no longer taking?

- Yes, specify: _____ [P11]
- No
- Unknown

Q6. Caregiver: Are there any medicines that the patient is taking more or less of?

- Yes, specify: _____ [P11]
- No
- Unknown

Q7. Caregiver: Do you have a list of the patient's current medicine information?

- Yes
- No [P11]
- Unknown

Q8. Caregiver: What medicine side effects do you watch for? (Check all that apply.)

- Allergic reactions [P10]
- Bladder changes (urinating more or less) [P10]
- Blood sugar changes [P10]
- Bowel changes such as loose stools (diarrhea) or hard stools (constipation) [P10]
- Dizziness or tiredness that affects daily activity [P10]
- Upset stomach (nausea) or throwing up (vomiting) [P10]
- Vital sign changes (heart rate, blood pressure, respirations, temperature, pain) [P10]
- Weight changes [P10]
- Other, specify: _____ [P10]
- Unknown [P10]

- Not applicable

Q9. Caregiver: Where do you keep the patient's medicine?

- Bathroom cabinet [P12]
- Bedside tables [P12]
- Kitchen cabinet [P12]
- Kitchen counter [P12]
- Pillbox [P12]
- Refrigerator [P12]
- Other, specify: _____ [P12]
- Unknown [P12]
- Not applicable

Q10. Caregiver: How much medicine does the patient have before needing a refill?

- 3 months or more
- 1 to 3 months
- 2 to 4 weeks [P13]
- 2 weeks or less [P13]
- Unknown [P13]
- Not applicable

Q11. Caregiver: Are there fewer than 2 refills for any medicine? (Look at the prescription label for the information on "number of refills before" date.)

- Yes [P13]
- No
- Unknown [P10]
- Not applicable

Q12. Caregiver: Does the patient take all medicines as ordered?

- Yes
- No [P10] Medication Adherence [CCG](#)
- Unknown [P10]
- Not applicable

Psychosocial Assessment

Q1. Caregiver: Does your child drink alcohol, use tobacco, or take drugs? (Check all that apply.)

- Drinks alcohol Alcohol Use [CCG](#)
- Smokes or uses other tobacco product Tobacco Use [CCG](#)
- Uses illegal drugs or abuses prescription drugs Substance Use [CCG](#) Substance-Related Disorders - Self-Care [CCG](#)
- No
- Unknown

Q2. Case Manager: Has a psychosocial assessment been completed?

- Yes
- No Start of Care Psychosocial Assessment, Pediatric [CCG](#)
- Unknown Start of Care Psychosocial Assessment, Pediatric [CCG](#)

Quality Measures

- Disease Management Quality Measures:
 - Information provided from this question may be requested by various accrediting organizations.

Plan of Care

Barriers

- B1.** Health literacy barrier
- B2.** Linguistic barrier
- B3.** Hearing impairment
- B4.** Vision impairment

Problems

P1. Language barrier present

- **Goal:** Language barrier resolved **N**
 - **Interventions:**
 - **ASSIST:** Identifying translation options and using the most practical and appropriate one(1)
 - **COORDINATE:** Call with patient and translation service and obtaining consent from patient to use translator(2)(3)
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **EDUCATE:** Ways to overcome language barriers(1)(4):
 - Speak slowly in normal tone.
 - Discuss one topic at a time.
 - Repeat and summarize often.
 - Use names instead of pronouns.
 - Use short, simple words and sentences.
 - Use interpretation or translation services.(5)
 - **COORDINATE:** Communication to providers to offer education materials in native language, whenever possible(5)(6)
 - **COORDINATE:** Communication between care team members and care plan reconciliation(7)(8)(9)
 - **COORDINATE:** Follow-up contact to evaluate language barrier status, referral status, and need for further assistance
 - **SEND:** What you need to know - Chronic condition management  (Spanish)

P2. Hearing impairment in child present

- **Goal:** Child's hearing maximized(10)(11) **N**
 - **Interventions:**
 - **ASSIST:** Identifying patient and caregiver's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **COORDINATE:** Communication with patient's caregiver for program continuation
 - **COORDINATE:** Providing information and instructions in writing whenever possible(11)
 - **ASSIST:** Appointment scheduling for evaluation of hearing impairment(12)
 - **ASSIST:** Informing patient's providers of hearing loss and preferred alternative communication method
 - **ASSIST:** Identifying resources for hearing aids, phone lines, and other needed equipment and training
 - **ASSIST:** Utilizing a sign language interpreter trained for medical settings who can assist during medical appointments, if appropriate
 - **EDUCATE:** Ways to work around hearing loss(12)(13):
 - Hearing aid or assistive device
 - Writing or drawing pictures
 - Reading lips
 - Technical aids (eg, TTY phone service or closed-captioning)
 - Gestures or sign language
 - **EDUCATE:** Use, care, and benefit of hearing aids, if appropriate
 - **EDUCATE:** Implications of hearing loss (eg, safety risks)
 - **EDUCATE:** Ways to prevent further hearing problems(12):
 - Avoid places with loud, persistent noises.
 - Keep environmental noises down when trying to listen.
 - Manage ear problems (eg, infection) quickly.
 - **EDUCATE:** Ways caregivers can help with communication(12)(13):
 - Devote full attention when speaking or listening to patient.
 - Make lip-reading easier by facing patient when speaking, talking slowly, and not covering mouth.
 - Ask patient to repeat back what they heard.
 - Provide other ways to communicate (eg, offering paper for writing).
 - **EDUCATE:** When to get help for hearing problems (eg, infection, hearing problems get worse)
 - **EDUCATE: Resource:** Deaf Linx: www.deaflinx.com
 - **COORDINATE:** Otolaryngologist referral, if indicated
 - **COORDINATE:** Audiologist referral, if indicated(12)
 - **COORDINATE:** Speech-language pathologist referral, if indicated
 - **COORDINATE:** Occupational therapist referral, if indicated
 - **COORDINATE:** Home care referral, if indicated
 - **COORDINATE:** Communication between care team members and care plan reconciliation(7)(8)(9)
 - **COORDINATE:** Follow-up contact to evaluate hearing status, referral status, and need for further assistance(14)(15)

P3. Hearing evaluation needed for child

- **Goal:** Child's hearing evaluated and self-care understood^N
 - **Interventions:**
 - **ASSIST:** Identifying patient and caregiver's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for hearing evaluation
 - **EDUCATE:** When to get help for hearing problems (eg, infection, hearing problems get worse)(12)(13)
 - **EDUCATE:** Implications of hearing loss (eg, safety risks)(11)
 - **COORDINATE:** Communication with patient's caregiver for program continuation
 - **COORDINATE:** Audiologist referral, if indicated(12)
 - **COORDINATE:** Speech-language pathologist referral, if indicated
 - **COORDINATE:** Home care referral, if indicated
 - **COORDINATE:** Communication between care team members and care plan reconciliation(7)(8)(9)
 - **COORDINATE:** Follow-up contact for results of hearing evaluation, referral status, and need for further assistance(14)(15)

P4. Vision impairment in child present

- **Goal:** Child's vision maximized(10)(16)^N
 - **Interventions:**
 - **ASSIST:** Identifying patient and caregiver's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **COORDINATE:** Communication with patient's caregiver for program continuation
 - **ASSIST:** Appointment scheduling with provider for evaluation of visual impairment(17)
 - **ASSIST:** Informing patient's providers of vision loss and patient's preference for communication
 - **ASSIST:** Identifying resources for visual aids (glasses, contact lenses) and other needed equipment and training
 - **ASSIST:** Obtaining written materials in braille, if indicated
 - **EDUCATE:** Ways to work around vision loss(13)(18):
 - Visual aids or assistive devices (glasses, contact lenses)
 - Voice options for electronics
 - Sighted companion
 - Cane and mobility training
 - **EDUCATE:** Implications of vision impairment(13)(18):
 - Interference with ADL and increased risk for falls and other injuries
 - Impact on psychological, emotional, and social well-being
 - **EDUCATE:** Ways to prevent further vision problems(16)(18):
 - Keep eyes healthy (nutrition support, washing hands before eye care, wearing protective eyewear).
 - Importance of evaluation for changes in vision
 - Manage eye problems (eg, infection) quickly.
 - **EDUCATE:** When to get help for vision problems (eg, infection, vision problems get worse)(13)(18)
 - **EDUCATE:** Importance of routine eye examinations
 - **COORDINATE:** Ophthalmologist referral, if indicated
 - **COORDINATE:** Occupational therapist referral, if indicated(13)
 - **COORDINATE:** Home care referral, if indicated
 - **COORDINATE:** Communication between care team members and care plan reconciliation(7)(8)(9)
 - **COORDINATE:** Follow-up contact to evaluate vision status, referral status, and need for further assistance(14)(15)

P5. Vision evaluation needed for child

- **Goal:** Child's vision evaluated and self-care understood(16)^N
 - **Interventions:**
 - **ASSIST:** Identifying patient and caregiver's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for visual evaluation
 - **COORDINATE:** Ophthalmologist referral, if indicated
 - **COORDINATE:** Home care referral, if indicated
 - **EDUCATE:** When to get help for vision problems (eg, infection, vision problems get worse)(13)
 - **EDUCATE:** Implications of vision loss (eg, safety risks)
 - **EDUCATE: Resources:**

- American Foundation for the Blind: www.afb.org
- National Federation of the Blind: www.nfb.org
- **COORDINATE:** Community services referral, as needed
- **COORDINATE:** Communication between care team members and care plan reconciliation(7)(8)(9)
- **COORDINATE:** Follow-up contact for results of vision evaluation, referral status, and need for further assistance(14)(15)

P6. Provider care for child needed

- **Goal:** Provider established to oversee child's healthcare needs(19)
 - **Interventions:**
 - **ASSIST:** Identifying patient and caregiver's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Identifying appropriate provider to oversee patient's care management plan
 - **ASSIST:** Identifying and scheduling appointment with primary care provider
 - **ASSIST:** Communication between multiple providers
 - **EDUCATE:** Purpose, role, and benefit of having provider oversee all of patient's care to prevent treatment gaps, delays, overlaps, and problems
 - **COORDINATE:** Primary care provider referral, if needed
 - **COORDINATE:** Care team meeting, if needed(9)(20)
 - **COORDINATE:** Follow-up contact to review outcome of provider visit, referral status, and need for further assistance(14)(15)

P7. Child's vaccinations not up to date, increasing infection risk

- **Goal:** Child's vaccinations up to date(21)(22)
 - **Interventions:**
 - **ASSIST:** Identifying patient and caregiver's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling
 - **ASSIST:** Identifying vaccines patient may need that are not contraindicated based on clinical status
 - **EDUCATE:** Purpose and benefits of vaccinations
 - **EDUCATE:** Recommended immunizations and timing of administration:
 - Centers for Disease Control and Prevention's vaccination schedule: [www.cdc.gov/vaccines\(22\)\(23\)\(24\)](http://www.cdc.gov/vaccines(22)(23)(24)
 - Vaccine schedules may need to be altered based on medications; check with the specialist provider for information.(25)
 - Possible side effects or complications associated with getting vaccinated
 - **EDUCATE:** Influenza vaccination annually(26):
 - Side effects are mild and last 1 or 2 days.
 - Common side effects with shot are sore, red, or swollen area where shot was given; fever; and aches.
 - Common side effects with nasal spray are mild and last 1 or 2 days; they include runny nose, headache, sore throat, and cough.
 - Allergic reaction rare and is usually caused by a chicken egg allergy
 - **COORDINATE:** Community services referral, as needed
 - **COORDINATE:** Vaccinations
 - **COORDINATE:** Follow-up to evaluate vaccination understanding, referral status, and need for further assistance(14)(15)
 - **SEND:** Coronavirus disease 2019 (COVID-19) vaccines 📄 (Spanish)
 - **SEND:** Pneumonia vaccine for children 📄 (Spanish)
 - **SEND:** Flu vaccine 📄 (Spanish)
 - **SEND:** Hepatitis A vaccine 📄 (Spanish)
 - **SEND:** Hepatitis B vaccine 📄 (Spanish)

P8. Healthy diet information needed for child

- **Goal:** Child's nutritional status optimized(27)
 - **Interventions:**
 - **ASSIST:** Identifying patient and caregiver's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for dietary review and self-care plan

- **EDUCATE:** Healthy body weight and height for child based on age[A](28)
- **EDUCATE:** BMI levels.(29) (See  BMI Calculator for more information.)
- **EDUCATE:** Dietary recommendations vary based on age and calorie needs(30):
 - Calorie needs are based on age, BMI, and health status.
 - Most calories should come from whole grains, vegetables, fruits, and low-fat dairy products.
 - Serving sizes are based on recommended calorie intake of child.
 - Vary sources of protein (eg, meat, eggs, seafood, nuts).
 - No more than 10% of calories from saturated fats
 - Adequate fluid intake[B]
- **EDUCATE:** Food safety (eg, avoid foods that may be choking hazard)
- **EDUCATE:** Oral hygiene
- **EDUCATE: Resources:**
 - Women Infants and Children: www.fns.usda.gov/wic
 - U.S. National Library of Medicine: www.nlm.nih.gov
 - U.S. Department of Agriculture MyPlate Plan: www.myplate.gov
- **EDUCATE: When to get emergency help:**
 - Respiratory impairment (eg, gasping for air, choking, cyanosis, chest pain, severe dyspnea)(31)
 - Dehydration (eg, breathing hard, weight loss, eyes sunken, fontanelles sunken, irritability, not responding to sounds or sights)(32)
 - Aspiration (eg, coughing, choking, hypoxia, wheezing)
- **EDUCATE: When to call the provider:**
 - Crying more for no reason or crying that cannot be soothed
 - Feeding problems
 - Urinary problems (eg, pain, frequency change, blood)
 - Bowel problems (eg, frequency change, blood)
 - Infection (eg, low-grade fever, sore throat, nausea, vomiting, or diarrhea)
 - Caregiver cannot take care of patient anymore.
- **COORDINATE:** Dietitian referral, if indicated(28)
- **COORDINATE:** Dentist referral, if indicated
- **COORDINATE:** Home care referral, if indicated
- **COORDINATE:** Community services referral, as needed
- **COORDINATE:** Social worker referral, as needed(33)
- **COORDINATE:** Care team meeting, if needed(9)(20)
- **COORDINATE:** Communication between care team members and care plan reconciliation(7)(8)(9)
- **COORDINATE:** Follow-up contact to review understanding of child's nutritional needs, referral status, and need for further assistance(14)(15)
- **SEND:** How to read food labels (Illustrated)  (Spanish)
- **SEND:** Intake and output diary (Illustrated)  (Spanish)

P9. Potential nutrition problems for child

- **Goal:** Nutritional problems in child resolved
 - **Interventions:**
 - **ASSIST:** Identifying patient and caregiver's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for dietary review and self-care plan
 - **ASSIST:** Creation of nutrition diary
 - **COORDINATE:** Laboratory testing, if indicated
 - **COORDINATE:** Calorie or fluid counting
 - **EDUCATE:** Nutritional recommendations for patient's health status
 - **EDUCATE:** Food and drug interactions potentially impacting nutritional status
 - **EDUCATE:** High BMI nutritional recommendations include(28):
 - Low-fat diet with lower-glycemic-load foods
 - Avoid sugary foods (eg, sodas, desserts).
 - Smaller portions to control caloric intake
 - Eat breakfast; avoid skipping meals.
 - **EDUCATE:** Low BMI nutrition recommendations include(34)(35):
 - Eat high-energy foods such as nuts, seeds, eggs, protein supplements, and fortified cereals.
 - Choose foods with high nutritional value (eg, fruits, green leafy vegetables, whole grain breads, and low-fat dairy products).
 - Avoid foods high in solid fats, added sugars, or alcohol.(36)

- Protein supplements: add to applesauce, puddings, milkshakes, soups, juices, and fruit smoothies, if indicated.
- Rely on food, not supplements, for nutritional needs. Supplements should be in addition to usual diet, not a replacement.
- Eat more often, such as 6 small meals rather than 3 large meals.
- Gradually increase portion size.
- Do not fill up on fluids before eating.
- Ensure high-calorie nutritious snacks go with patient to day care or school.
- Caregivers supervising child's eating should encourage child to snack.
- Weight loss too severe: caloric intake may need to increase to 120% to 150% of recommended daily allowance.
- **EDUCATE:** Weight monitoring in children includes(37):
 - Keep diary of food intake and daily weight.
 - Weigh consistently (eg, same time, scale, clothes).
- **COORDINATE:** Developmental status assessment with appropriate government organizations (eg, county school system)(38)
- **COORDINATE:** Community services referral, as needed(38)
- **COORDINATE:** Dentist referral, if indicated
- **COORDINATE:** Dietitian referral, if indicated(38)
- **COORDINATE:** Home care referral, if indicated
- **COORDINATE:** Pharmacy medication support, if indicated
- **COORDINATE:** Protective services or legal authorities, as needed
- **COORDINATE:** Rehabilitation therapy referral, if indicated
- **COORDINATE:** Social worker referral, as needed(33)
- **COORDINATE:** Specialist referral, if indicated
- **COORDINATE:** Speech-language pathologist referral, if indicated(39)
- **COORDINATE:** Care team meeting, if needed(9)(20)
- **COORDINATE:** Communication between care team members and care plan reconciliation(7)(8)(9)
- **COORDINATE:** Follow-up to evaluate child's nutritional status, referral status, and need for further assistance(14)(15)
- **SEND:** Meal diary (Illustrated)  (Spanish)
- **SEND:** Intake and output diary (Illustrated)  (Spanish)
- **SEND:** Weight loss eating plan  (Spanish)
- **SEND:** Tips for getting your child to eat more  (Spanish)
- **SEND:** Eating to keep up your energy  (Spanish)
- **SEND:** Eating when it is hard to swallow  (Spanish)
- **SEND:** How to read food labels (Illustrated)  (Spanish)
- **SEND:** Weight diary (Illustrated)  (Spanish)

P10. Medication information needed

- **Goal:** Medication information understood
 - **Interventions:**
 - **ASSIST:** Identifying patient and caregiver's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for discussion on appropriate medication
 - **ASSIST:** Creation of current and reconciled medication list
 - **ASSIST:** Identifying activities that promote medication adherence
 - **EDUCATE: Reason for taking medication(40):**
 - Expected effects if the medication is working, and expected time for effects to occur
 - How long medication will likely need to be taken
 - What happens if medication is not taken as prescribed
 - **EDUCATE:** Information on all medications(40)(41)(42)(43):
 - Medication purpose, dose, effects/side effects, and considerations (eg, managing missed doses, interactions with food/other drugs)
 - Name of provider who ordered medication
 - Administration techniques
 - Side effects, related management, and when to contact provider
 - **Get emergency help:** Adverse or allergic reactions (eg, wheezing, chest tightness, fever, itching, bad cough, blue skin color, hemorrhage, seizures, swelling of face, lips, tongue, or throat)
 - Common side effects include gastrointestinal changes, rash, dizziness, and headache.
 - Do not stop long-term medications abruptly due to risk of withdrawal symptoms.

- Medication safety (eg, proper storage and disposal)(44)
- Monitoring (eg, vital signs, laboratory checks) needed while taking medication
- Talk to provider before taking any over-the-counter medications, vitamins, minerals, or herbal or dietary supplements.
- Carry a list of medications at all times, particularly when visiting healthcare providers.
- **COORDINATE:** Clinical monitoring appropriate to medication use (eg, laboratory tests)
- **COORDINATE:** Communication between patient and pharmacy (for obtaining prescriptions, setting up home medication delivery, review for drug-drug interactions)(45)(46)[N](#)
- **COORDINATE:** Communication between care team members and medication list reconciliation.(43)(47) See Medication Reconciliation Tool [SR](#) for more information.[N](#)
- **COORDINATE:** Obtaining medication organizer or reminder tools (eg, pillbox)
- **COORDINATE:** Home care referral, if indicated
- **COORDINATE:** Social worker referral, as needed(33)
- **COORDINATE:** Follow-up to evaluate adherence or problems with medication, provide additional education and support, and identify need for further assistance(14)(15)
- **SEND:** Managing your medicines (Illustrated)  (Spanish)

P11. Medication changes present

- **Goal:** Medication changes understood and medication taken as ordered
 - **Interventions:**
 - **ASSIST:** Identifying patient and caregiver's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for evaluation of medication problems (clarification of dose or regimen)
 - **ASSIST:** Creation of current and reconciled medication list
 - **ASSIST:** Identifying activities that promote medication adherence
 - **ASSIST:** Communication with treating provider when another provider (eg, emergency or specialty provider) prescribes new medication
 - **EDUCATE: Reason for taking medication(40):**
 - Expected effects if the medication is working, and expected time for effects to occur
 - How long medication will likely need to be taken
 - What happens if medication is not taken as prescribed
 - **EDUCATE:** Information on all medications(40)(41)(42)(43):
 - Medication purpose, dose, effects/side effects, and considerations (eg, managing missed doses, interactions with food/other drugs)
 - Name of provider who ordered medication
 - Administration techniques
 - Side effects, related management, and when to contact provider
 - **Get emergency help:** Adverse or allergic reactions (eg, wheezing, chest tightness, fever, itching, bad cough, blue skin color, hemorrhage, seizures, swelling of face, lips, tongue, or throat)
 - Common side effects include gastrointestinal changes, rash, dizziness, and headache.
 - Do not stop long-term medications abruptly due to risk of withdrawal symptoms.
 - Medication safety (eg, proper storage and disposal)(44)
 - Monitoring (eg, vital signs, laboratory checks) needed while taking medication
 - Talk to provider before taking any over-the-counter medications, vitamins, minerals, or herbal or dietary supplements.
 - Carry a list of medications at all times, particularly when visiting healthcare providers.
 - **COORDINATE:** Communication between care team members and medication list reconciliation.(43)(47) See Medication Reconciliation Tool [SR](#) for more information.[N](#)
 - **COORDINATE:** Communication with pharmacy (eg, home delivery, change in medication form, help with drug-drug or food-drug problems)(45)(46)[N](#)
 - **COORDINATE:** Follow-up contact for evaluation of medication understanding, referral status, and need for further assistance(14)(15)
 - **SEND:** Managing your medicines (Illustrated)  (Spanish)

P12. Medication safety information needed

- **Goal:** Medication safety understood and medication safety implemented(48)
 - **Interventions:**
 - **ASSIST:** Identifying patient and caregiver's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up

- **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
- **ASSIST:** Appointment scheduling with primary care provider when another provider (eg, emergency provider or other specialist) prescribes new medication
- **EDUCATE:** Medication safety instructions(43):
 - Discard outdated medication.
 - Do not share prescription medication.
 - Store out of reach of children and in childproof containers.
 - Fill all prescriptions at the same pharmacy.
 - Ensure proper storage (in original containers and with other needs such as refrigeration or protection from sunlight).
 - Talk to provider before taking any over-the-counter medications, vitamins, minerals, or herbal or dietary supplements.
- **EDUCATE:** Administration techniques
- **EDUCATE:** De-prescribing or cessation of medications safely (eg, pharmacy-supported, provider monitoring planned) (49)
- **COORDINATE:** Communication with pharmacy for medication interaction review
- **COORDINATE:** Communication between care team members and medication list reconciliation.(9)(47) See Medication Reconciliation Tool  SR for more information.
- **COORDINATE:** Follow-up contact for evaluation of medication understanding, referral status, and need for further assistance(14)(15)
- **SEND:** Managing your medicines (Illustrated)  (Spanish)

P13. Medication refills needed

- **Goal:** Medications refilled
 - **Interventions:**
 - **ASSIST:** Identifying patient and caregiver's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
 - **ASSIST:** Obtaining refills in lieu of an appointment
 - **ASSIST:** Identifying resources to pay for medications, if needed
 - **ASSIST:** Working with pharmacy for filling prescriptions (eg, changing pills to liquids, identifying financial support, delivery)(42)
 - **EDUCATE:** Importance of getting medications refilled prior to running out (1 to 2 weeks before)
 - **EDUCATE:** Pharmacy assistance programs, such as:
 - Centers for Medicare and Medicaid Services (CMS): www.medicare.gov/part-d
 - Medicine Assistance Tool: <https://medicineassistancetool.org>
 - NeedyMeds: www.needymeds.org
 - Individual company websites
 - **COORDINATE:** Communication with pharmacy (eg, home delivery, change in medication form, help with drug-drug or food-drug problems)
 - **COORDINATE:** Pharmacy medication support, if indicated
 - **COORDINATE:** Follow-up to evaluate adherence or problems with medication, provide additional education and support, and identify need for further assistance(14)(15)
 - **SEND:** Managing your medicines (Illustrated)  (Spanish)

Evidence Summary

Quality standards: Standards indicate that a needs assessment should be completed for every patient to identify and resolve potential communication barriers, including vision impairments, hearing impairments, and languages spoken other than English. Programs should have a process for following up with patients on the status of referrals.(50)

Quality standards: Standards indicate that a needs assessment should be completed for every patient to identify and resolve potential communication barriers, including vision impairments, hearing impairments, and languages spoken other than English. Programs should have a process for following up with patients on the status of referrals.(50)(51)

Outcomes: Several studies support the role of the pharmacist in medication reconciliation to identify and resolve pharmacy-related problems and improve outcomes (eg, prevention of medication-related adverse events, discontinuation of inappropriate medication).(52)

Evidence: A prospective cohort study of 2655 hospitalized patients after discharge found that 24% of hospital medication changes were not followed, 30% of changed doses were filled with the incorrect dose, 27% of new medications were not filled, and 12% of discontinued medications were filled. Risk factors for medication change problems included increased out-of-pocket costs, not having medications filled before hospitalization, and discharge to a long-term care facility.(53)

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Footnotes

[A] Percentiles may be obtained by using clinical growth charts, such as those available through the Centers for Disease Control and Prevention at www.cdc.gov/growthcharts. Percentiles rank the status of a child by showing what percentage of the reference group of the same age and sex the child would equal or exceed. They do not measure how well a child is growing because growth rate is measured over time and has a high level of variation between individuals. [A in Context Link 1]

[B] An estimated minimum fluid intake of 50 to 60 mL/kg per day may be appropriate for most children. Considerations for fluid intake include the patient's age, activity level, and medical status.(28)(39) [B in Context Link 1]

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